

METROPOLITAN CARDIOLOGY ASSOCIATES

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PATIENT MEDICAL CHART

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1. PATIENT DEMOGRAPHICS

Patient Name: Robert James Mitchell	Date of Birth: March 4, 1958
MRN: MCA-004821	Age: 67 years
Sex: Male	Gender Identity: Male
SSN (last 4): ****-7713	Marital Status: Married
Address: 218 Birchwood Ln, Staten Island, NY 10301	Phone: (718) 555-0247
Emergency Contact: Sandra Mitchell (Wife)	EC Phone: (718) 555-0391
Preferred Language: English	Race/Ethnicity: White, Non-Hispanic
Primary Care Physician: Dr. Angela Fiorentino, MD	PCP Phone: (718) 555-0122

2. INSURANCE & COVERAGE

Primary Insurance

Payer: UnitedHealthcare	Plan Type: Medicare Advantage
Plan Name: AARP Medicare Advantage Choice PPO	Plan ID: H0062-001-000
Member ID: UHC887134521A	Group Number: 82734-MA-NY
Medicare Beneficiary ID: 4BK9-WX3-KL72	Part A Effective: April 1, 2021
Part B Effective: April 1, 2021	Coverage Effective: January 1, 2025
Copay (Specialist): \$45	Copay (Inpatient): \$325/day (days 1-5)
Deductible: \$0 (met)	Out-of-Pocket Max: \$6,700/year
Prior Auth Required: Yes — Cardiac Procedures	Auth Contact: 1-800-555-0162

Secondary Insurance

None on file.

3. ACTIVE PROBLEM LIST

ICD-10	Diagnosis	Onset	Status
R00.1	Bradycardia, unspecified	Oct 2025	Active
I10	Essential (primary) hypertension	2014	Active

ICD-10	Diagnosis	Onset	Status
E11.9	Type 2 diabetes mellitus without complications	2018	Active
E78.5	Hyperlipidemia, unspecified	2016	Active
Z82.49	Family history of ischemic heart disease	N/A	Active

4. CURRENT MEDICATIONS

Medication	Dose	Frequency	Indication
Metoprolol succinate	25 mg	Daily	Hypertension
Lisinopril	10 mg	Daily	Hypertension
Atorvastatin	40 mg	QHS	Hyperlipidemia
Metformin	500 mg	BID	Type 2 DM
Aspirin	81 mg	Daily	Cardiovascular prophylaxis

Note: Metoprolol was reviewed as a possible contributor to bradycardia. Cardiology determined dose is low and not the primary etiology. No medication changes made at this time.

5. ALLERGIES & ADVERSE REACTIONS

Allergen	Reaction	Severity
Penicillin	Urticaria, angioedema	Moderate
Iodine contrast (IV)	Nausea, flushing	Mild

6. VISIT NOTES — CARDIOLOGY

Visit Date: October 8, 2025 | Attending: Dr. Marcus Tran, MD, FACC | Type: New Patient Consult

Chief Complaint:

Referral from PCP for incidentally noted bradycardia on routine EKG. Patient denies chest pain, dyspnea, syncope, presyncope, or palpitations.

History of Present Illness:

67-year-old male with hypertension, T2DM, and hyperlipidemia presenting after PCP noted HR of 46 bpm on routine EKG during annual visit. Patient is entirely asymptomatic. He reports no episodes of dizziness, lightheadedness, near-fainting, or loss of consciousness. He exercises regularly (walks 2 miles daily) without limitation or symptoms. No exertional intolerance. No nocturnal symptoms reported by patient or spouse. Sleep quality described as good. No recent illness, thyroid disease, or electrolyte disturbances known.

Physical Examination:

HR: 44 bpm, regular	BP: 138/82 mmHg (Right arm, seated)
RR: 14 breaths/min	SpO2: 97% on room air
Temp: 98.4 F	Weight: 192 lbs BMI: 27.4
Cardiovascular: Regular rhythm, bradycardic. No murmurs, rubs, or gallops.	Respiratory: CTA bilaterally. No wheezes or crackles.
Extremities: No edema. Peripheral pulses 2+ bilaterally.	Neuro: Alert and oriented x3. No focal deficits.

Assessment & Plan:

- Bradycardia — HR 44-48 bpm. Asymptomatic. No functional impairment reported. Etiology not yet established. Metoprolol reviewed; low-dose, unlikely sole cause.
- Order 30-day ambulatory Holter monitor to capture rhythm burden, pauses, and any correlation with symptoms if they develop.
- Order TSH, BMP, CBC to exclude metabolic causes.
- Return in 4-6 weeks with monitoring results.
- Patient counseled to report immediately if syncope, near-syncope, dyspnea on exertion, or significant fatigue develops.

Electronically signed: Dr. Marcus Tran, MD, FACC — October 8, 2025 at 3:47 PM

Visit Date: November 12, 2025 | Attending: Dr. Marcus Tran, MD, FACC | Type: Follow-Up

Interval History:

Patient returns to review Holter monitor results. He reports no new symptoms in the interim period. No syncope, near-syncope, dizziness, or exertional limitations. Lab results reviewed below.

Laboratory Results (November 2025):

Test	Result	Reference	Interpretation
TSH	2.4 mIU/L	0.4-4.0	Normal
Sodium	139 mEq/L	136-145	Normal
Potassium	4.1 mEq/L	3.5-5.0	Normal
Calcium	9.2 mg/dL	8.5-10.5	Normal
Creatinine	1.0 mg/dL	0.6-1.2	Normal
Hemoglobin A1c	7.1%	<7.0%	Slightly elevated

Holter Monitor Summary (30-Day, November 2025):

Monitoring Duration: 29 days, 14 hours	Mean HR: 46 bpm
Min HR: 38 bpm (sleeping)	Max HR: 104 bpm (exertion)
Pauses > 2.5 sec: 12 events recorded	Longest Pause: 3.1 seconds (nocturnal)
SVT Runs: None detected	VT/VF: None detected
AV Block detected: None (1:1 conduction throughout)	Symptom Correlation: No patient-triggered events logged

Assessment & Plan:

- Sinus bradycardia with nocturnal pauses up to 3.1 seconds. No AV block identified. No arrhythmia correlated with symptoms — patient logged zero symptom events during the monitoring period.
- Metabolic workup negative. TSH normal. No reversible cause identified.
- Patient remains fully asymptomatic with no functional limitation.
- Patient counseled to report any episodes of syncope, near-syncope, sustained dizziness, or significant fatigue. Pacemaker implantation to be considered pending further evaluation.
- Continue current medication regimen. Metoprolol dose stable, no change at this time.
- Follow-up in 3 months or sooner if symptoms develop.

Electronically signed: Dr. Marcus Tran, MD, FACC — November 12, 2025 at 2:18 PM

7. DIAGNOSTIC IMAGING & TESTING REPORTS

Electrocardiogram (EKG) — October 8, 2025

Ordering Physician: Dr. Marcus Tran, MD, FACC	Interpreting Physician: Dr. Marcus Tran, MD, FACC
Facility: Metropolitan Cardiology Associates	Date Performed: October 8, 2025
Indication: Bradycardia evaluation	Rate: 44 bpm
Rhythm: Normal sinus rhythm with bradycardia	PR Interval: 172 ms (normal)
QRS Duration: 88 ms (normal)	QT/QTc: 440/377 ms
Axis: Normal — 42 degrees	ST Segment: No ST elevation or depression

Interpretation: Sinus bradycardia at 44 bpm. Normal PR interval. Normal QRS morphology and duration. No bundle branch block. No evidence of ischemia or prior infarct. No AV block identified. No Wolff-Parkinson-White pattern.

Electronically signed: Dr. Marcus Tran, MD, FACC — October 8, 2025

Transthoracic Echocardiogram (TTE) — November 5, 2025

Ordering Physician: Dr. Marcus Tran, MD, FACC	Interpreting Physician: Dr. Linda Zhao, MD
Facility: Metropolitan Cardiology Imaging	Date Performed: November 5, 2025
Indication: Bradycardia, structural assessment	Image Quality: Adequate
LVEF: 58% (normal)	LV End-Diastolic Diameter: 4.9 cm (normal)
LV Wall Motion: Normal, no regional abnormality	LA Size: 3.8 cm (mildly enlarged)
Mitral Valve: Trace mitral regurgitation, no stenosis	Aortic Valve: Trileaflet, no stenosis or regurgitation
Tricuspid Valve: Trace tricuspid regurgitation	Pericardium: No effusion
RVSP Estimate: 28 mmHg (normal)	Diastolic Function: Grade I diastolic dysfunction (impaired relaxation)

Impression: Preserved left ventricular systolic function. No structural cause for bradycardia identified. Mild left atrial enlargement, likely related to hypertension. Grade I diastolic dysfunction consistent with age and comorbidities. No significant valvular disease.

Electronically signed: Dr. Linda Zhao, MD — November 6, 2025

30-Day Holter Monitor Report — November 1-29, 2025

Ordering Physician: Dr. Marcus Tran, MD, FACC	Interpreting Physician: Dr. Marcus Tran, MD, FACC
Monitor Type: Zio Patch XT (iRhythm)	Wear Duration: 29 days, 14 hours
% Analyzable: 97.2%	Symptom Events Logged: 0 (zero)
Mean HR: 46 bpm	Min HR: 38 bpm
Max HR: 104 bpm (exertion)	Pauses > 2.5 sec: 12 events
Longest Pause: 3.1 sec (0143 hrs, Nov 12)	Pause Context: All nocturnal, during sleep

AV Block: None. 1:1 conduction throughout.	SVT Burden: None detected
Atrial Fibrillation: None detected	Ventricular Ectopy: Rare isolated PVCs (<0.1%)

Interpretation: Persistent sinus bradycardia with mean rate of 46 bpm over the monitoring period. Nocturnal pauses up to 3.1 seconds identified, consistent with increased vagal tone during sleep. No AV block identified. No atrial fibrillation or flutter. No symptomatic arrhythmia. Zero patient-triggered events logged during the 29-day period. No symptom-rhythm correlation can be established.

Electronically signed: Dr. Marcus Tran, MD, FACC — November 11, 2025

8. PLANNED PROCEDURE

Planned Procedure: Permanent Cardiac Pacemaker Implantation	CPT: 33207 — Single chamber pacemaker, ventricular
Unit 1: Single chamber pacemaker — ventricular lead	Surgeon: Dr. Marcus Tran, MD, FACC
Facility: Metropolitan Cardiology Associates	Payer: UHC AARP Medicare Advantage PPO
PA Status: Pending — Submitted November 14, 2025	Auth #: Pending

Clinical Indication Summary:

- Diagnosis: Non-reversible bradycardia (R00.1) — mean HR 46 bpm, nocturnal pauses up to 3.1 seconds on 30-day Holter monitoring.
- Metabolic workup negative: TSH, BMP, CBC all within normal limits. No reversible cause identified.
- EKG: Sinus bradycardia at 44 bpm. No AV block. Normal QRS morphology.
- Echo: Preserved LVEF 58%. No structural cause for bradycardia.
- Holter (Nov 1-29, 2025): 12 nocturnal pauses, longest 3.1 seconds. Zero patient-logged symptom events.
- Metoprolol reviewed as potential contributor — low dose (25 mg), deemed unlikely sole etiology by cardiology.

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